GEEVI EVIDENCE BRIEF - PUBLISHED SOURCE - ATTRIBUTED SUMMARY

VIALE-A - randomized-trial appraisal brief

Source type: Phase 3 randomized controlled trial

Publication date: 2020-08-13

Citation: DiNardo CD, Jonas BA, Pullarkat V, et al. Azacitidine and Venetoclax in Previously Untreated Acute Myeloid Leukemia. N Engl J Med. 2020;383(7):617-629. doi:10.1056/NEJMoa2012971. PMID: 32786187. Trial: NCT02993523. https://pubmed.ncbi.nlm.nih.gov/32786187/

Question: In previously untreated AML unsuitable for standard induction, what benefits, harms and applicability limits were reported for azacitidine plus venetoclax compared with azacitidine plus placebo?

Population: Original 2020 intention-to-treat analysis: 431 patients, 286 combination and 145 control; median age 76. Patients were previously untreated and considered ineligible for standard induction because of age 75 or older, coexisting conditions, or both.

Intervention: Azacitidine plus venetoclax, described for evidence appraisal only; no dosing instructions are included.

Comparator: Azacitidine plus placebo.

Outcomes: Primary endpoint in the original publication: overall survival Complete remission and composite complete remission Adverse events and serious adverse events

Finding 1: At median follow-up 20.5 months, median overall survival was 14.7 versus 9.6 months; hazard ratio for death 0.66, 95% confidence interval 0.52-0.85. Composite complete remission was 66.4% versus 28.3%. Grade 3 or higher febrile neutropenia occurred in 42% versus 19%; serious adverse events in 83% versus 73%. Benefits and harms should be appraised together.

Limitations: The original publication's abstract and trial identifier were verified; the full paper and supplements were not appraised in this pass. The study population was unsuitable for standard induction. Results cannot be generalized automatically to all newly diagnosed AML, an individual molecular subgroup, or a patient without a documented fitness assessment. Funded by AbbVie and Genentech. The brief is a starter appraisal, not a systematic review or current treatment recommendation. The current registry lists 443 actual enrollment, a later 2021 data cutoff and last update posted2026-06-11. These are distinct from the original 2020 paper's 431-person analysis; denominators and versions must not be merged.

Applicability: A patient-linked research question should record treatment history, clinician-assessed fitness, relevant comorbidity and unresolved information before judging applicability. In Geevi, applicability is an attributed clinician assessment. The existence of this evidence does not make either fictional patient eligible or prescribe a regimen. Use the trial record to check eligibility definitions and provenance. On 2026-10-11 the record was active, not recruiting; do not present it as an open enrollment option.

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Original Geevi evidence brief, not the original article or a treatment recommendation.

Sources verified: 2026-10-11. Full-text appraisal and a current literature review remain necessary.